

Patient:

Referring Physician:

DOB:

Age:

Gender: F

Ref#:

Hosp#:

Provider Group :

Date of Service:

Date Received:

Case

Date Reported:

A copy of this report will be faxed to:

FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

A. - C.) LEFT BREAST, ADDITIONAL ANTERIOR MARGIN, AND LEFT AXILLARY SENTINEL LYMPH NODE SIMPLE MASTECTOMY (INCLUDING RE-EXCISION OF ANTERIOR MARGIN) AND SENTINEL NODE BIOPSY:

- Invasive ductal carcinoma, Nottingham grade 3.
 - Size: 2.6 cm in greatest dimension.
 - Location: 11:00.
- No in situ component identified.
- Margins:
 - Invasive carcinoma is widely free of all margins (at least 1 cm away).
 - Additional anterior margin consists of benign skin and subcutaneous tissue.
- No lymphovascular invasion identified.
- Two sentinel lymph nodes, negative for carcinoma (0/2).
- Uninvolved breast parenchyma with focal fibroadenomatoid changes.
- Benign skin and nipple with small intraductal papilloma.

UID: 302E69E0-39F0-4CDB-97F9-A6CDB83D7233
TCGA-AC-A6IW-01A-PR

Redacted



COMMENT: Breast carcinoma biomarker studies will be repeated on a representative section of tumor (block A7) due to the focal weak to strong reactivity for ER and PR in <1% tumor cells seen previously on core biopsy. The results will follow in an addendum.

PATHOLOGIC TUMOR STAGING SYNOPSIS:

Type and grade (invasive): Invasive ductal carcinoma, Nottingham grade 3.
Type and grade (in situ): N/A.
Primary tumor: pT2.
Regional lymph nodes: pN0(i-)(sn).
Distant metastasis: N/A.
Stage: IIA.
Lymphovascular invasion: Not identified.
Margin status: R0, negative.

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Invasive Breast Cancer Tumor Staging Information

AJCC Cancer Staging Handbook, 7th Ed., and CAP Protocol (revised June 2012).

Previous pathology specimens

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SPECIMEN IDENTIFICATION

Procedure/specimen type:

Laterality:

Lymph node sampling:

Simple mastectomy with re-excision of anterior margin.

Left breast.

Sentinel lymph node biopsy.

INVASIVE CARCINOMA TUMOR CHARACTERISTICS

Histologic type:

Tumor site:

Tumor size:

Tumor focality:

Histologic grade (Nottingham Score):

Tubule formation:

Nuclear pleomorphism:

Mitotic rate:

Invasive ductal carcinoma.

11:00.

2.6 cm in greatest dimension.

Unifocal.

3 of 3 (Nottingham score 9 of 9).

3 of 3.

3 of 3.

3 of 3.

Lymphovascular invasion:

Macroscopic and microscopic extent of tumor:

Not identified.

Benign skin and nipple.

DUCTAL CARCINOMA IN SITU (DCIS):

Not identified.

MARGINS

Invasive carcinoma:

Widely free of all margins (at least 1 cm away).

LYMPH NODES

Total lymph nodes examined

Number of lymph nodes involved

Size of largest metastatic deposit:

Extranodal extension:

2 (sentinel nodes).

0.

N/A.

N/A.

PATHOLOGIC STAGING:

Primary Tumor (pT):

Regional lymph nodes (pN):

Distant metastasis (pM):

AJCC Stage:

pT2.

pN0(i-)(sn).

N/A.

IIA.

ANCILLARY STUDIES:

Estrogen receptor:

Progesterone receptor:

HER2:

Ki-67:

Negative (less than 1% positive cells, weak intensity).

Negative (less than 1% positive cells, strong intensity).

Negative (score 0).

High (70% positive cells).

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Signed by

Source of Specimen:

- A. Breast; left
- B. Sentinel lymph node; left axillary
- C. Breast; left additional anterior margin

Clinical History/Operative Dx:

Malignant neoplasm of breast (female, left).

Intraoperative Diagnosis:

- B. Left axillary sentinel lymph node : No tumor cells identified

Gross Description:

A. Part A is designated as left breast - stitch = lateral. Initially received in the fresh state for possible Is an 862 gram left simple mastectomy, 26.8 cm medial-lateral, 20.8 cm inferior-superior, and 4.0 cm posterior-anterior. The overlying ellipse of wrinkled, tan-brown skin is 26.3 x 13.0 cm with a medial-inferior orientated, darkened brown, wrinkled areola, 4.8 cm in diameter with a prominent 1.3 cm nipple. A double suture marks the lateral apex of the skin. A palpable mass is appreciated in the upper inner quadrant (approximating 11:00). The surgical margins are now differentially inked as follows:

Superior superficial: Blue,
Inferior superficial: Orange,
Posterior: Black.

Included within the container are two unoriented strips of wrinkled, tan-brown skin measuring 11.7 x 2.7 x 0.7 cm and 10.3 x 2.4 x 1.3 cm each.

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The breast is serially sectioned perpendicularly through the medial-lateral long axis to reveal a well-demarcated, soft to rubbery, light gray, lobulated tumor mass measuring upwards of 2.6 x 2.5 x 2.3 cm (slabs 6-7, from medial). This tumor is placed grossly 1.0 cm from the superior superficial, 2.4 cm from the deep and greater than 5.0 cm from the inferior superficial, most medial and most lateral surgical margins. Along the medial periphery of the lesion is an embedded twisted radiologic marker.

The remaining cut sections of the breast demonstrates admixed yellow-tan fibrofatty tissue without additional discrete nodularity. No lymph node tissue is appreciated along the lateral periphery. Sectioning through the remaining two strips of skin demonstrate unremarkable fibrofatty cut surfaces. Representative sections are submitted.

Cassette summary:

- A1) nipple,
- A2) medial and lateral extension of skin,
- A3) upper inner quadrant, slab 3,
- A4) upper inner quadrant, slab 5, adjacent to tumor,
- A5) upper inner quadrant, tumor and superior superficial surgical margin, slab 6,
- A6-A7) upper inner quadrant, tumor and superior superficial surgical margin, contiguous sections, slab 6,
- A8) upper inner quadrant, deep margin adjacent to tumor, slabs 6-7,
- A9) upper inner quadrant-central superior, adjacent to tumor, slab 8,
- A10-A11) lower inner quadrant, slabs 4 and 6, respectively,
- A12-A13) lower outer quadrant, slabs 9 and 11, respectively,
- A14-A15) upper outer quadrant, slabs 9 and 11, respectively,
- A16-A17) representation of separate strips of skin and subcutaneous fat, two pieces each.

B. Part B is left axillary sentinel lymph node (frozen section). Initially received in the fresh state for frozen section/touch prep analysis is a 6.0 x 3.0 x 1.0 cm pad of yellow-tan fatty soft tissue. Initial sectioning reveals a single fatty pink and tan lymph node candidate, 2.0 cm in greatest dimension. Two touch preparation slides are forwarded for microscopic evaluation. Further examination of the fat reveals possible additional lymph node tissue up to 1.0 cm in greatest dimension. The largest lymph node is entirely submitted for routine histology with a sentinel node protocol performed in B1 and B2, and the additional/residual lymph node tissue candidacy is submitted in B3 and B4 for routine histology.

C. Part C is additional anterior margin - left. Initially received in the fresh state is a 15 gram elliptical excision of wrinkled, tan-brown skin and subcutaneous fat, 11.1 x 2.6 x 1.5 cm. The margins are submitted unoriented; the possible outer margin is now marked blue and the remaining surgical margins are marked black. Sectioning demonstrates glistening, admixed yellow-tan fibrofatty tissue without discrete nodularity or grossly unusual areas of change suggestive of residual or invasive tumor component. Representative section are submitted for microscopic evaluation in C1-C5 (six pieces).

Microscopic Description:

A. - C.) Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered. The sentinel nodes (part B) were evaluated by H&E levels and cytokeratin immunostain. Controls are appropriate.

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Criteria	6/13/13	Yes	No
Diagnosis Discrepancy			✓
Primary Tumor Site Discrepancy			✓
HIPAA Discrepancy			✓
Prior Malignancy History			✓
Distal/Synchronous Primary Noted			✓
Case is (circle):	QUALIFIED	DISQUALIFIED	
Reviewer: Initials	Kmi	Date Reviewed:	6/13/13

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END OF REPORT (FINAL)